

**Question on notice no. 2328**

**Portfolio question number: DHDA SQ25-003155**

**Supplementary Budget estimates**

**Community Affairs Committee, Health, Disability and Ageing Portfolio**

**Senator Alexander Antic:** asked the Department of Health, Disability and Ageing on 12 December 2025—

The recently released NCIRS "Surveillance of adverse events following immunisation in Australia, COVID-19 vaccines, 2022"<sup>1</sup> provides that: "Of the 18,398 AEFI reports for COVID-19 vaccines in 2022, 160 (0.9%) reported an outcome of death following vaccination, of which only one was determined as likely to be linked to vaccination, as assessed by the TGA and reviewed by an expert panel."

Those reported deaths include:

Six children (age 0-17) where the median time from vaccination to death in the children was 4.5 days; and

Five 18-29 years olds where the median time from vaccination to death was 3 days.

According to the WHO causality assessment manual<sup>2</sup>, this short temporal association is highly suggestive of causality, and statistically very unlikely to be due to chance.

The Vaccine Safety Investigation Group (VSIG) working document (TGA FOI 4029-063) makes clear these deaths should have all been reported to VSIG, on account of the fact that young people were at such low risk from COVID, that any death from a COVID vaccine should have had the potential to change the favourable benefit-risk balance of the vaccine.

13. Were any of the six children who died in 2022 referred to VSIG? If not, why not?

14. Were any of the five 18-29 years olds who died in 2022 referred to VSIG? If not, why not?

15. Do there exist causality assessment reports for each of the children where a fatal adverse event from the covid vaccines has been reported? Please provide copies (redacted for privacy) . If they do not exist, why not?

16. Do there exist causality assessment reports for each of the 18-29 years olds where a fatal adverse event from the covid vaccines has been reported? Please provide copies (redacted for privacy) . If they do not exist, why not?

17. Why were only 2 of the fatal cases from 160 in 2022 referred to VSIG?

18. How many of the 1053 deaths reported on the DAEN system have had causality assessments?

19. How many of the 1053 deaths reported on the DAEN system been referred to VSIG, and where can these assessments and reports be accessed?

**Answer —**

Please see attached answer.

# Senate Committee: Community Affairs Committee

## QUESTION ON NOTICE

December Supplementary Budget Estimates 2025-2026

Outcome: 1 - Health Policy, Access and Support

PDR Number: SQ25-003155

**Question Subject:** Covid VSIG

**Type of Question:** Written

**Senator:** Alex Antic

### Question:

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**Answer:**

13 & 14. The preamble to these questions does not accurately represent the WHO publication '*Causality assessment of an adverse event following immunization (AEFI)- User manual for the revised WHO classification second edition 2019 update*'<sup>1</sup> (the WHO user manual), the role the Therapeutic Goods Administration's (TGA's) Vaccine Safety Investigation Group (VSIG), or the principles of pharmacovigilance.

The WHO user manual provides the following rationale for causality assessment of AEFI: '*Appropriate action(s) must be taken to respond promptly, efficiently, and with scientific rigour to vaccine safety issues. This will minimize adverse effects to the health of individuals and entire populations and in turn help to maximize the benefits of immunization programmes. Causality assessment of adverse event following immunization (AEFI) is thus a vital component of AEFI risk assessment, decision-making and the initiation of action.*'

Contrary to the statement in the preamble, the WHO user manual states '*The fact that a vaccine was administered within a reasonable time period of the occurrence of an event does not automatically suggest that the vaccine caused or contributed to the event*'.

The WHO user manual sets forth causality assessment as a systematic review of data about an AEFI, and correlation with available scientific evidence. The manual recommends use of a systematic causality assessment checklist in which the timeframe between administration of a vaccine and onset of the adverse event (AE) is one of a number of factors considered. It is therefore not credible, nor in line with the WHO causality assessment framework for AEFI, to assert that a temporal association alone is highly suggestive of causality.

The TGA's VSIG Work Instruction, available at TGA FOI 4029-063<sup>2</sup>, confirms the purpose of the VSIG is '*to provide independent specialist immunisation (and other relevant) expertise to assist the TGA and OHP [Office of Health Protection] to investigate and manage AEFI and vaccine safety signals of concern*'.

Therefore, individual AEFIs are not "*reported to VSIG*" as described in the preamble. Rather, a VSIG may be convened to provide specialist immunisation expertise to assist the TGA in investigating and/or managing AEFIs or vaccine safety signals, should certain criteria be met. These criteria include when:

- an AEFI of concern or a safety signal of concern is identified by the TGA or OHP
- the TGA and OHP agree that the AEFI or signal has the potential to change the favourable benefit-risk balance of the vaccine in a National or State immunisation program or could threaten public confidence in vaccine safety, and
- the case(s) is/are considered eligible for assessment and/or investigation.

The VSIG Work Instruction further specifies that an AEFI of concern is a serious AEFI, that is unexpected, and without an obvious non-vaccine cause. It also defines a serious AEFI as an event that results in death, is life-threatening, requires inpatient hospitalisation or prolongation of existing hospitalisation, results in persistent or significant disability/incapacity, or is a congenital anomaly/birth defect.

All reports of serious AEFI are reviewed by the TGA. This review is designed to assess whether the serious AEFI report represents an emerging safety concern, and to determine whether a regulatory response is necessary based on the weight of evidence. The TGA's review considers the strength of the evidence for a link between vaccination and the AEFI using a standardised process based on the WHO's AEFI causality assessment guide. As part of this process, the TGA may convene a VSIG to access specialist expertise in investigating or

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<sup>1</sup> <https://iris.who.int/server/api/core/bitstreams/2a09fc09-127c-47e7-b289-e20523ded26b/content>

<sup>2</sup> <https://www.tga.gov.au/sites/default/files/2023-01/foi-4029-06.PDF>

managing a regulatory response to an AEFI, provided the criteria for convening a VSIG are met.

Therefore, noting the information above, convening a VSIG is one of a number of tools available in causality assessment for a serious AEFI. It is not correct to suggest that all AEFI reports where an outcome of death is reported will automatically meet the criteria to convene a VSIG.

A serious AEFI may not meet criteria for convening a VSIG if, for example, the individual case report lacks sufficient information at the time of reporting, or if the report describes a known AE with no new vaccine safety information identified. It is important to emphasise that a VSIG is not a death investigation group, and has no role in determining the cause of death for an individual or diagnosing the medical cause of an individual's symptoms.

It is also important to emphasise that investigation of a single case report of a serious AEFI, including cases with death reported as the outcome, is just one part of a broad and comprehensive system of pharmacovigilance for vaccines registered in Australia.

This system relies on many elements, including

- periodic statistical analysis of aggregated AE reports,
- close liaison with state and territory immunisation coordinators and public health units,
- regular consultation with specialist bodies such as the National Centre for Immunisation Research and Surveillance
- assessing active surveillance data from the AusVaxSafety system,
- reviewing published medical literature and mainstream media,
- close liaison with sponsors of registered vaccines, and
- ongoing monitoring of international regulatory counterparts to detect and investigate emerging vaccine safety signals.

Individual case reports of serious AEFI are considered within the broader global context of pharmacovigilance and post-market regulation of vaccines, with any resulting safety investigation examining both clinical data at the individual level and evidence at the population level, alongside consideration of regulatory options to mitigate risks at the population level.

The TGA's publicly available Database of Adverse Event Notifications (DAEN) includes 9 reports of AEFI with outcome reported as death in people aged less than 18 years following COVID-19 vaccination. None of these reports met the criteria to convene a VSIG.

While the TGA does not comment on specific details for individual cases, reasons for not proceeding to VSIG for these cases include that specific AEs were known for that vaccine and did not constitute new safety information, identification of alternative causes of death by coroners and/or treating healthcare professionals, or insufficient information being available. This is in line with both the purpose and criteria for convening a VSIG. Two reports of serious AEFI with death as an outcome in individuals aged under 18 years did progress to broader safety investigations undertaken by the TGA in late 2022 and early 2023. These investigations considered available population-level evidence for the association between the vaccine and AE, as well as wider regulatory or vaccine program implications.

One fatal report in an 18–29-year-old who died in 2022 was referred to VSIG.

Since 2021 there have been 15 VSIGs convened to review serious AEs reported following COVID-19 vaccination, reviewing the details of over 50 cases, noting not all cases reviewed by the VSIG had an outcome recorded as death.

The first panel reviewed reports of anaphylaxis, with the findings publicly available at: [www.tga.gov.au/safety/safety-monitoring-and-information/safety-alerts/astrazeneca-chadox1-s-covid-19-vaccine-no-evidence-increased-risk-anaphylaxis](http://www.tga.gov.au/safety/safety-monitoring-and-information/safety-alerts/astrazeneca-chadox1-s-covid-19-vaccine-no-evidence-increased-risk-anaphylaxis)

Following this, public statements for all VSIGs convened during the rollout of COVID-19 vaccines were published in the TGA's COVID-19 vaccine safety reports, accessible at: <https://www.tga.gov.au/news/covid-19-vaccine-safety-reports>. The specific reports detailing outcomes of VSIGs are:

- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-07-04-2021](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-07-04-2021)
- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-14-04-2021](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-14-04-2021)
- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-21-04-2021](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-21-04-2021)
- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-28-04-2021](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-28-04-2021)
- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-10-06-2021](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-10-06-2021)
- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-08-07-2021](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-08-07-2021)
- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-29-07-2021](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-29-07-2021)
- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-26-08-2021](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-26-08-2021)
- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-09-12-2021](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-09-12-2021)
- [www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-27-01-2022](http://www.tga.gov.au/periodic/covid-19-vaccine-weekly-safety-report-27-01-2022)
- [www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-weekly-safety-report-19-05-2022](http://www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-weekly-safety-report-19-05-2022)
- [www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-weekly-safety-report-16-06-2022](http://www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-weekly-safety-report-16-06-2022)
- [www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-safety-report-23-09-2022](http://www.tga.gov.au/news/covid-19-vaccine-safety-reports/covid-19-vaccine-safety-report-23-09-2022)

Regardless of the outcome of causality assessment of an individual case report of serious AEFI, all AE reports where death is recorded as the outcome are included in statistical analyses to identify and investigate safety signals at the population level. The TGA does not remove any reports of death from the DAEN. The TGA remains responsive to new information about any serious AEFI, particularly those with an outcome recorded as death, regardless of when the original report was received.

15 & 16. The TGA does not disclose details of individual cases to ensure patient privacy and safeguard the sensitive health information contained in those reports. The majority of the information contained within the documents related to AE reports with a fatal outcome is highly sensitive health information relating to deceased persons. Even with any names redacted, an individual may still be reasonably identifiable to their family members or members of the public. Reports may also include information about any pre-existing medical conditions.

Disclosure of personal information may reduce public willingness to submit reports and thereby substantially prejudice the TGA's ability to regulate therapeutic goods. To encourage patients and practitioners submit AE reports the TGA accepts these reports on the understanding that an individual's personal information will be properly safeguarded, including the details of those reporting an AE

Publicly available information about VSIGs convened during the roll out of COVID-19 vaccines are available via the TGA's COVID-19 vaccine safety reports website at: [www.tga.gov.au/news/covid-19-vaccine-safety-reports](http://www.tga.gov.au/news/covid-19-vaccine-safety-reports)

17. This has been addressed above in answer to questions 13 and 14.

18. Please refer to response to questions 13 and 14.

19. Please refer to response to questions 13 and 14.

The TGA has answered a number of previous SQoNs related to COVID-19 vaccines, deaths and safety signal investigation. Please see:

- SQ24-000230 – COVID-19 Vaccine autopsies (published 15 April 2024)
- SQ24-000218 – Does the TGA track reported deaths and adverse events by batch number? (published 12 April 2024)
- SQ24-000234 – COVID-19 Vaccines - TGA FOI 4263 - Determinations on Cause of Death (published 12 April 2024)
- SQ24-000233 – COVID-19 Vaccines - TGA FOI 4205 - Database of Adverse Event Notifications (DAEN) (published 12 April 2024)
- SQ23-002226 – Reporting of COVID vaccine deaths (published 18 January 2024)
- SQ23-001101 – Follow up of reported death in the Australian (published 1 September 2023)
- SQ23-000139 – Causality assessments of the death of a 7 and 9 years old (published 29 August 2023)
- SQ23-001074 – Temporal association between the rollout of the vaccine and excess deaths (published 10 August 2023)
- SQ23-001086 – FOI 4217 - Deaths of children under 18 (published 27 July 2023)
- SQ23-001134 – Deaths of children following covid vaccine (published 26 July 2023)